ETHICS CLASS 3
JANUARY 29
CONFIDENTIALITY AND PRIVILEGED COMMUNICATION

AGENDA

Clinical Training Q's

Client Right and Informed Consent

- **Time with partners to work on your consent forms**

Faith, Fidelity and Responsibility

Confidentiality

- Rational
- Exceptions
- Duty to warn, protect and report
- Panel activity

Privileged Communication

ELC: SITE SELECTION

Go to the ELC:

<u>ONLY AFTER YOU HAVE RECIEVED AND ACCEPTED AN OFFER!</u>

1. **Select a site** (or **'suggest a site'** for new sites)
2. **Select a supervisor** (or start an application for new supervisors)
3. **Supervision Dates Form** (first day of Spring Semester (4/27/26) and go to August of 2026 (8/6/2027)
4. **Professional Practice Agreement**

(this form is signed by you, your supervisor and me)

REVIEW: WHEN IS IT APPROPRIATE TO REFER A CLIENT? (ACA, 2014, A.11 A,C)

- Client no longer needs services (c)
- Client is unlikely to benefit (c)
- Client is at risk of being harmed (c)
- Client does not pay fees (c)
- Client or someone the client knows puts the counselor in jeopardy of harm (c)
- Counselor lacks competency in a clinical area to be of professional assistance to a client (outside of the clinician's scope of practice) (a)

REVIEW: WHEN WOULD IT BE INAPPROPRIATE TO REFER ACCORDING TO ACA CODE (A.11 B, D, A.12?)

- Discrimination
- Differences in values, attitudes, beliefs, behavior or personality preferences
- Abandonment considerations (when client is in crisis, and other services are not set up; termination services have not occurred)

CLIENT RIGHTS

Client rights

- Counselors are responsible for **safeguarding clients' welfare** from the initiation of the counseling relationship and extend through termination of the relationship. see NBCC Booklet for clients
  - This does not mean you work 24-7 but that you have plans in place for example you do not give out your cell number or be available outside of work hours for your client. You provide clients to call a 24-hours mental health line, 911 or go to an emergency room when you are not available.
- **Counselors have a fiduciary relationship with their clients** (this is a legal term meaning you act in their benefit). Clients are not there for your benefit, you are there for them.

Awareness of personal needs and motivations

- As counselors, we get the chance to work with and care for people we never thought we would. It is a high calling and privilege to love people from varied backgrounds.

- However, your history and reactions will come into play. They can become entangled in the counseling relationships. This is called a **countertransference reaction**. See Case of Lynn on pg. 87
- Personal motivations in keeping clients, Video example 4.1

The counselor as a person and a professional

- Counselors must be aware of the influence of their own personality and needs
- Personal needs of counselors based on unresolved personal conflicts:
  - a need to tell people what to do
  - a desire to take away all pain from clients
  - a need to have all the answers and to be perfect
  - a need to be recognized and appreciated
  - a tendency to assume too much responsibility for the changes of clients
  - a fear of doing harm, however inadvertently

  - Corey et al., Issues and Ethics - Chapter 2 (1)

Transference

- Transference is the process whereby clients project onto their therapists past feelings or attitudes they had toward significant people in their lives
- Transference: the “unreal” relationship in therapy
  - Counselors need to be aware of their personal reactions to a client’s transference
  - All reactions of clients to a therapist are not to be considered as transference
  - Ethical issue is dealing appropriately with transference

Cory et al. Issues and Ethics - Chapter 2 (2)

Recognize your own reactions

- **Transference**: The feelings, attitudes experiences your client projects onto you
- **Countertransference** – YOUR feelings, attitudes and experiences that come up for you in your work with a client (you need to be aware of these)
  - Desire to nurture?
  - Desire to be liked?
  - Reaction to an issue/topic that is presented

Discussion: How might countertransference reactions be dangerous in a therapeutic relationship

Countertransference check ins

- being overprotective with a client
- treating clients in kind ways
- rejecting a client
- needing constant reinforcement and approval
- seeing yourself in your clients
- developing sexual or romantic feelings for a client
- giving advice compulsively
- desiring a social relationship with clients

Examples of ways counselors might engage in ethically questionable behavior that would be difficult to detect and, thus, difficult to enforce:

- Prolonging the number of counseling sessions to satisfy the counselor's emotional or financial needs
- Being unaware of countertransference reactions to a client, student, or supervisee and thereby inadvertently increasing uncooperative behavior and thwarting growth
- Imposing values, goals, or strategies on clients that are not congruent with their cultural background
- Using techniques or strategies that are comfortable for the counselor rather than those that are aimed at helping clients achieve their therapeutic goals
- Practicing with little enthusiasm or tolerating boredom and apathy
- Ignoring self-care practices and thus not being able to practice effectively

Mandated clients

- Mandated clients may refuse services we must explain the consequences of their refusal (ACA, A. 2. e). We also must be accurate, honest and objective in the evaluations (ACA, 2014, C. 6. b.)
- Counselors must work to provide a safe context with clear guidelines and transparency.
- Court mandates often require disclosure to the court. Make it clear to the client if the purpose of the meeting is evaluation that will be disclosed or counseling.
- Consider viewing counseling and evaluation and two separate process for these clients, involving two different clinicians "Reporting on counseling attendance and progress toward reaching counseling goals set by clients is different from evaluating clients for issues such as fitness for parenting, ability to work after an injury, a mental condition, or progress toward eligibility for parole."

- See p. 477 for disclosure example for a mandated client.
- Written informed consent is required to move from a therapeutic role to an evaluative role.

Treatment plans and techniques

- Establishing goals and treatment plans should be done jointly with clients. What are their hopes for counseling? What do they want to focus on?
- Specialized techniques should be grounded in empirical research (ACA, 2014, C.7.a.) and counselors should ensure they have completed training and are aware of diversity considerations.
- Experimental techniques should be discussed and may require written disclosure
- Potential risks and benefits should be disclosed to the client

Termination reasons

- Client has reached their goals and no longer needs assistance
- Client is not benefiting or is being harmed by counseling
- Client does not pay the fees agreed upon
- The counselor is in jeopardy of being harmed by the client or someone the client has a relationship with
- The counselor and client moves

- Sometimes termination is unexpected, in this case a plan should be in place to safeguard the client's welfare. It is a good idea to have a plan on who would notify your clients/who has access to client records in a severe health or emergency situation

Avoiding abandonment (inappropriate termination, ACA, 2014, A.12)

- Do not terminate when the client is in crisis, when the client is angry or has an objection to counseling.
- Honestly discuss with the client your intention to terminate and the specific reason.
- Give the client an opportunity to make changes that would be necessary to continue the relationship.
- Give written notice to the client that the relationship will be terminated.
- Offer the client one to three termination sessions.
- Give your client up to three good referrals.
- Give your client places to contact in the event of emergencies.
- Place in the client's file a summary of your interactions with the client that are related to the termination issue. Do not transfer that document to another individual or entity without an express written request from the client.
- Give the client plenty of time to find another mental health professional. If more time is requested, allow it.
- Transfer records to the new mental health professional promptly.

Counseling Techniques

- Is my approach a good fit for this client?
  How might I need to adjust?
- Counselor have an obligation to select appropriate counseling strategies or techniques.
  - Any techniques that could be considered experimental or unusual should be used very cautiously
  - Strategies should fit your counseling style and be tailored to the specific needs of the client
  - Critically, counselors should not use techniques that might cause harm to a client

Federal laws such the Health Insurance Portability and Accountability Act (HIPAA) and state laws require specific written consent to treatment be obtained from clients, prior to counseling services being rendered.

Getting informed consent from clients

- Securing the client's informed consent is both a legal and an ethical obligation.
  - Demonstrates respect for a client's autonomy and self-determination
  - Integral to the formation of an effective therapeutic alliance
- The more that clients understand about how counseling works, the more they are likely to benefit from the therapeutic process.

Informed consent continued

- Written contracts for informed consent are commonly referred to as *disclosure statements*.
- These documents disclose to clients the nature of the counseling relationship into which they are entering.
- Serve as legal contracts that are signed by both the client and the counselor.
- It can be difficult to achieve a balance between providing too little information and giving clients so much information that they feel overwhelmed or intimidated.
- **video example 4.3**

Diversity considerations

- Diversity considerations:
  - Counselors must be sensitive to the fact that informed consent, as traditionally conceptualized, may not be respectful of people of all cultures.
  - Using language that clients can understand is equally important with clients who are children, adults who are not well educated, and clients whose first language is not English (translation tools, advocating for various languages being provided for consent documents, etc.)

Informed consent is an ongoing process that should be conducted both orally and in written form.

ACTIVITY

Read the case of Ibrahim, and review the NBCC Booklet for clients

- Why is informed consent so important
- Practice explaining the process of informed consent to a client
- How might you make this process interesting for a client
- What essential things will you want to go over from the document with a client
- How would you include information about client rights?

8 areas that, if not handled properly, could lead to legal problems (see examples in text, p. 100)

- Failing to include required HIPAA elements
- Guaranteeing an outcome as a result of counseling
- Guaranteeing privacy with no exceptions
- Agreeing to fee arrangements different from what was intended
- Touching without implied or actual permission
- Misrepresenting credentials
- Failing to communicate the nature of counseling
- Neglecting to warn client about possible stigma

Informed consent should include:

1. the purposes, goals, techniques, procedures, limitations, and potential risks and benefits of services;
2. the counselor's qualifications;
3. information about termination and referrals;
4. emergency protocols and crisis resources;
5. the role of technology (and a backup plan if tech fails);
6. the implications of diagnosis and intended use of testing and reports;
7. fees and billing information;
8. confidentiality and its limitations;
9. the right to obtain records and actively participate in ongoing counseling plans (see HIPPA requirements to include regarding disclosure on p. 98);
10. and the right to refuse recommended services or modality change and be advised of the consequences of refusal.

Informed Consent Documents

- In groups of 2-3 look at the counseling disclosure agreements on pg. 472-475, compared to one of the examples or your own in Discovery and the list provided. Some are missing required elements.
  - What items are missing?
  - What similarities and differences do you notice?
  - Is there anything you might add if you were creating an informed consent document

Faith and Fidelity

The professional ethical principle of fidelity: Keeping promises is related to the topics of privileged communication, privacy and confidentiality

Watson relates the the theological virtue of **faith** with fidelity

WHAT IS FAITH?

Aquinas wrote it involves “believing someone and believing something”

> *Have you ever had someone believe in you when you couldn’t see it? What was the impact of this experience?*

Hebrews 11 highlights the mystery in faith, being a part of something much bigger than can be understood in this life. How do

Now faith is ***confidence in what we hope for and assurance in about what we do not see***. (Hebrews 11:1-3, NIV)

Faith, fidelity and responsibility

- Watson (2018) argues many theologians connect faith with trust. She says, “The virtue of faith in practice can be evidence by acts of fidelity and trustworthiness toward one’s community” (Watson, 2018, p. 42). Christian therapists can work to exhibit a virtue of faith not only in their spiritual and personal lives, but professionally as well. According to Watson, therapists **live out faith through building trust in relationships with clients and colleagues**. Studies over the last 25 years show trust is an essential ingredient in psychological and relationship health (Furrow et al., 2019).

- While a general understanding of faith or trust in the counseling room is different from having faith in God, security and trust can be learned from being in relationship with God.

- Fidelity (same root as fiduciary) is: fulfilling a responsibility of trust in the counseling relationship by **honoring commitments and keeping promises**. Counselors strive to be faithful to the promises they make such as keeping clients disclosers confidential” (p. 11).

How does God see us?

You are loved (Jeremiah 31:3)
"I have loved you with an everlasting love""

You are cared for (Naham 1:7)
"God is a refuge in times of trouble, He cares for those who trust in him"

You are delighted in (Zephaniah 3:17)
"He will take great delight in you, he will quiet you with his love, he will rejoice over you with singing"

How might God's example to us guide us in caring for clients?

What do you hope your clients experience in their counseling relationship with you?

Cultivating faith

Watson recommends two critical components for the Christian in developing faith: **contemplative prayer and church involvement.**

"For the mental health professional (student)...developing habits of prayers is as essential as food and water for nourishing the soul and sustaining trust and belief in God during the rigors of graduate school, academic and life and professional practice" (Watson, 2018, p. 58)

Involvement in Christian community, regular worship, participation in the sacraments...**remind us** of the reality of God as creator, redeemer and sanctifier...and provide opportunity to **realign and affirm** our relationship with our creator. (Westberg, 2015, as cited in Watson)

Faith integration

"we uphold the ethical principle of fidelity not only because we have to but because it has become an essential part of our personal and professional character, guiding our attitudes, activates and relationships".

(Watson, 2018, p. 46-47)

Fidelity and responsibility

Fidelity- "honoring commitments and keeping promises including fulfilling one's responsibilities of trust in professional relationships" (ACA, 2014)

Which promises and responsibilities come to mind for you that are important to honor as counselors?

Counseling dispositions and behaviors (CCS-R 2d)

2. D		**Knowledge & Adherence to Site Policies**	**Demonstrates an understanding & appreciation for *all* counseling site policies & procedures.**	Demonstrates consistent adherence to *all* counseling site policies & procedures, including strong attendance and engagement.	Demonstrates adherence to most counseling site policies & procedures, including strong attendance and engagement.	Demonstrates inconsistent adherence to all counseling site policies & procedures, including attendance and engagement.

CONFIDENTIALITY AND PRIVILEGED COMMUNICATION

Key terms in counseling related to fidelity and respecting client autonomy

Privacy – Broadest term, includes client has a right to privacy of their information and the rights of an individual to decide which information about themselves is shared with others.

Confidentiality – confidentiality implies an explicit contract or promise not to reveal anything about a client except under certain circumstances agreed to by both parties.

Privileged communication – A legal term that protects clients from having confidential communications with their counselors disclosed in a court of law without their permission (Law and courts)

Court must prove that disclosure is vital for the wellbeing of society

Confidentiality is one of the most fundamental of all professional obligations in counseling.

Confidentiality (1 of 2)

- Helping professionals assume that an assurance of confidentiality is an indispensable requirement for effective counseling.
  - Without this assurance, many clients would not feel safe to openly discuss the intimate and personal aspects of their lives or would not seek counseling.
- Clients need to know that they can trust their counselors to respect their privacy
  - **A counselor's confidentiality pledge is the cornerstone on which this trust is built**.

Confidentiality (2 of 2)

- *Fidelity*: being faithful and keeping promises
  - One of the moral principles on which confidentiality rests
  - Accordingly, one of the most important promises that counselors make to clients is that they will keep the secrets shared in counseling.

Ethical Standards and Confidentiality

- Confidentiality is the only specific ethical issue to which an entire section of the *Code of Ethics* (ACA, 2014) is devoted.
- Section B: Confidentiality and Privacy emphasizes the client's right to privacy in the counseling relationship.
  - Exceptions and limitations to confidentiality are addressed
  - Because exceptions to confidentiality may be mandated by legal as well as ethical considerations, the standards are carefully written to minimize conflicts between law and ethics

When more than one person attends the counseling appointment

(groups, family therapy, interpreter present, mass trauma events, emergencies…)

Confidentiality cannot be guaranteed when more than one client is in the room

- The family system or the couple becomes the identified client unless another agreement has been established in the informed consent.
- Informed consent is important to discuss before counseling begins to ensure all parties understand the unique limits in family/couples counseling.
- Consider the case of Haruto on pg.127 and video example 5.2 (“can you keep a secret”)

Children 13-18

- RCW 71.34.530
- Any minor **thirteen years or older** may request and receive outpatient treatment without the consent of the minor's parent.

- RCW 70.02.130
- ...if the patient is a minor and is authorized to consent to health care without parental consent under federal and state law, **only the minor may exercise the rights of a patient** under this chapter as to information pertaining to healthcare to which the minor lawfully consented.

Important ages in WA State

- Outpatient Mental Health- 13
- Outpatient Substance Abuse Tx -13
- STD Testing & Tx-14
- Birth Control Services-any age
- Abortion Services-any age
- Prenatal Care Services-any age
- Sexual activity 16

see: https://doh.wa.gov/you-and-your-family/sexual-and-reproductive-health/state-laws

Family initiated treatment: For parents who need to initiate treatment for their kids 13+

Inpatient

The adolescent may provide notice in writing of intent to leave at any time.

The facility must date the notice and it must be sent to the adolescent's attorney and parent.

For substance abuse treatment, the notice may be sent to adolescent's parent, only with the adolescent's written consent.

RCW 71.34.510

Outpatient

The parent may seek outpatient mental health services for the adolescent for up to twelve sessions in a three-month period without consent from the adolescent.

Thereafter the adolescent must give his/her consent for further treatment if the professional determines that the adolescent needs treatment in a "less restrictive setting" including partial hospitalization or intensive outpatient treatment, the parent or adolescent may request such treatment in which case, the professional must convene a treatment review every 30 days after treatment begins.

Also a notice shall be provided within 24 hours to an independent reviewer from the Washington Health Authority who will determine within 45 days of first treatment if further treatment is necessary.

RCW 71.34.650

Limits to confidentiality and privilege according to WA law (RCWs) and the ACA (2014)

ROI

If the client brings a lawsuit against the counselor/subpoena from the secretary of health in response to a complaint

Child abuse/vulnerable adult reporting

Involuntary hospitalization

Imminent danger to the client themselves (threats to hurt themselves ACA, 2014, B.2.a) **or anyone else** including a life-threatening disease that is putting others in danger (ACA, 2014, B.2.a, c)

WA Law, Privileged Communication

Duty to warn others when a client poses a danger to other (may include life threatening diseases-see B.2.c.)

Duty to protect the client if they pose a danger to themselves or others

Duty to report suspected abuse or neglect of children or other persons presumed to have limited ability to care for themselves (vulnerable adults)

Duty to Warn

1976 Tarasoff vs University of California

- Counselors are charged with **violence prevention**
- You must assess for harm that clients have the POTENTIAL to act on
  - Poddar became obsessed with another student. He was mentally ill. He told a doctor about his obsession and desire to kill Tatiana Tarasoff. The doctor called the police, the police called him in, he said he wouldn't act. Poddar then did kill Tarasoff. Her family sued the University. Went to the supreme court and decided that the doctors could be liable for not protecting Tatiana. Before this there was an assumption that you couldn't control the actions of another.
  - Therapist must take **reasonable steps to care and protect the intended victim**. One of these steps may be to warn the potential victim, notify the police or whatever other steps may be necessary to protect the victim.
  - CA has a law that counselors are immune from liability when a client expresses to harm another, and the therapist should inform the victim and the police.

Shaub, 2023

Washington State case

- Volk v. Demeerleer
  - Demeerleer (hx of severe mental illness) murdered his girlfriend Rebecca.
  - Ashby was the psychiatrist
  - No hx of violence but rageful thoughts.
  - April 2010 Ashby saw him and reported SI but no plan to act, Ashby reports stability
  - July DeMeerleer kills girlfriend Rebecca and one of her sons
  - Rebecca's mother sues Ashby's clinic

  - Ashby says in court Demeerleer never identified who he might hurt or <u>any homicidal ideation</u>
  - The opposing argument was that Ashby did not provide a thorough enough assessment and monitoring, psychiatrist could have gotten family involved, scheduled more appointments etc.

Washington State

Supreme court said:

"We hold that Ashby and DeMeerleer shared a special relationship and that special relationship required Ashby to act with reasonable care, consistent with the standards of the mental health profession to protect the foreseeable victims of DeMeerleer"

A therapist's duty is NOT limited to a duty to warn in WA. **The duty to protect can arise even if the client does NOT express an intention to harm a reasonably identifiable victim.**

These kinds of cases can go to court to be argued/discussed. This case was settled on 12/19/17. It never went to trial. We don't know what would have happened, but it didn't get thrown out.

Interesting case. You can look up this article: **The Therapist and the Murderer: A psychiatrist and a lawyer analyze Volk v. DeMeerleer. Published in:** NW Lawyer, Feb2018, **By:** Berner, Jon;Preston, Patrick J. NW Lawyer; Feb2018, Vol. 72 Issue 1, p16-26, 11p

Threats to do Harm Legal Precedents (see Discovery)

Tarasoff Case

- duty to warn of harm to self or others
- duty to protect

Bradley Case

- duty not to negligently release a dangerous client

Jablonski Case

- duty to commit a dangerous individual

Hedlund Case

- extends duty to warn to anyone who might be near the intended victim and who might also be in danger

Jaffee Case

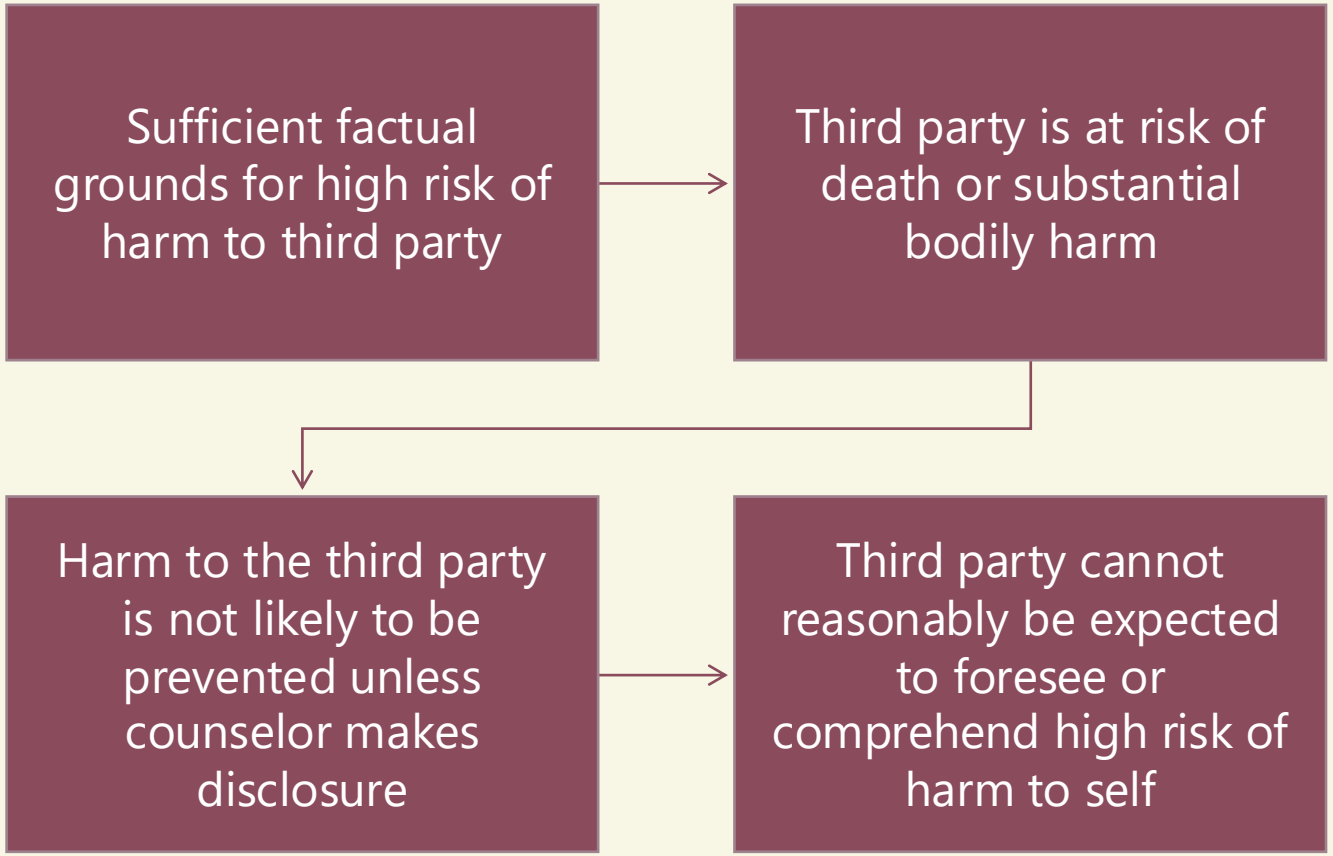
- communications between licensed psychotherapists and their clients are privileged and therefore protected from forced disclosure in cases arising under federal law

Guidelines for Implementing Duty to Warn Requirements

- Get informed consent
- Plan ahead
- consult, consult, consult!
- Develop contingency plans
- Obtain professional liability insurance
- Involve the client
- Obtain a detailed history
- Document in writing
- Implement procedures to warn

Corey et al., (2015). Issues and Ethics in the Helping Profession

Ethical Guidelines for Disclosure of a client's HIV Status



If all these are met, consult with the client's primary care medical provider. There is a process in WA state.

Duty to protect

Be prepared for high-risk cases

- Get supervision
- Become familiar with the procedures at your site for reporting an individual who is at risk of suicide/self harm-https://welevelupwa.com/mental-health/rickys-law/

- Have a safety plan in place
- Be aware of inpatient facilities in your area

- Video example 5.3 “don’t tell my parents”

If a client is at risk of hurting themselves, you must take necessary steps to prevent harm

This could look like-

- notifying individuals with whom the client lives
- notifying the parents of the client
- notifying client's medical providers
- Exploring voluntary inpatient options with the client
- Calling the police or designated crisis responder (DCR) (988 or 911)

  https://www.hca.wa.gov/assets/program/county-crisis-line-phone-numbers.pdf
- calling an ambulance to take the client to the hospital emergency room for evaluation

Death and dying act

- WA state has a death and dying act that allow clients who have less than 6 months to live request lethal doses of medication (70.245 RCW, passed on November 4, 2008)
- ACA code of ethics allow counselors the option of confidentiality
- Seek supervision and appropriate professional and legal advice if faced with this situation

Mandated Clients

- Are usually required to sign documents waiving their privacy
- Counselors should carefully explain the limits of privacy within the counseling relationship

Duty to Report (children and vulnerable adults)

REPORTING CHILD ABUSE

https://www.dcyf.wa.gov/safety/report-abuse

King County: 1-800-609-8764

Pierce County: 1-888-713-6115

Snohomish County: 1-866-829-2153

Age of consent in WA

Washington State will **not** prosecute based on age if someone has sex with:

•Someone **16 and over** (Chapter 9A.44 RCW). 16 is the age of consent in WA state

•Someone **14-15** if you are less than 4 years older (RCW 9A.44.79).

•Someone **12-13** if you are less than 3 years older (RCW 9A.44.76)

•Someone **under 12** if you are less than 2 years older (RCW 9A.44.073)

see: https://doh.wa.gov/you-and-your-family/sexual-and-reproductive-health/state-laws

What is Child Abuse and Neglect?

According to Washington State

- **RCW 26-44-020 (12)** defines abuse and neglect as injury, sexual abuse, sexual exploitation, negligent treatment or maltreatment of a child by any person under circumstances which indicate that the child's health, welfare, and safety is harmed. Abuse and neglect does NOT include the physical discipline of a child as defined in
- **RCW 9A.16.100** -It is the policy of this state to protect children from assault and abuse and to encourage parents, teachers, and their authorized agents to use methods of correction and restraint of children that are not dangerous to the children. However, the physical discipline of a child is not unlawful when it is reasonable and moderate and is inflicted by a parent, teacher, or guardian for purposes of restraining or correcting the child. Any use of force on a child by any other person is unlawful unless it is reasonable and moderate and is authorized in advance by the child's parent or guardian for purposes of restraining or correcting the child.

  The following actions are presumed unreasonable when used to correct or restrain a child: (1) Throwing, kicking, burning, or cutting a child; (2) striking a child with a closed fist; (3) shaking a child under age three; (4) interfering with a child's breathing; (5) threatening a child with a deadly weapon; or (6) doing any other act that is likely to cause and which does cause bodily harm greater than transient pain or minor temporary marks. The age, size, and condition of the child and the location of the injury shall be considered when determining whether the bodily harm is reasonable or moderate. This list is illustrative of unreasonable actions and is not intended to be exclusive.

Who is a Vulnerable Adult?

Washington State Department of Social and Human Services

A vulnerable adult is defined in WA law as:

- a person 60 years of age or older who lacks the functional, physical, or mental ability to care for him or herself;
- an adult with a developmental disability per 71A.10.020;
- an adult with a legal guardian per 11.88 RCW;
- an adult living in a long-term care facility (an adult family home, boarding home or nursing home);
- an adult living in their own or family's home receiving services from an agency or contracted individual provider; or an adult self-directing their care per law (74.39.050 RCW)

Reporting Child Abuse and Neglect

- Washington State Law requires *all* mandatory reporters to report suspected child abuse and neglect (RCW 26.44). For more information on reporting child abuse and neglect, look at https://www.dcyf.wa.gov/safety/report-abuse
- or call: 1-866-END HARM (1-866-363-4276)
- Learn more about mandatory reporting: https://www.dcyf.wa.gov/safety/mandated-reporter

Keep in mind

- Every decision involves both a yes and a no
- Decisions to breach confidentiality are nuanced and numerous. Consult consult consult. Is disclosure? required? Who should make the disclosure? Which individuals should be informed? Should the client be involved? How do you talk about disclosures with clients?

Privileged communication is a legal concept that is closely related to confidentiality but is different.

Privileged Communication (1 of 2)

- Privileged communication laws protect clients from having confidential communications with their counselors disclosed in a court of law without their permission.
  - Means that a judge cannot order a counselor to reveal information in court that has been recognized by law as privileged.

Asserting Privilege

- A statutory privilege belongs to clients rather than to counselors.
- If you or your records are subpoenaed it is up to your client to assert the privilege so that you will not have to disclose the information.
- If a client cannot be located when counselors are asked to disclose privileged information, the counselor has an obligation to assert the privilege on behalf of the client.
- Counselors should secure the advice of an attorney if they are put in the position of asserting a client's privilege.

Exceptions to Privileged Communication (1 of 2)

- Glosoff et al. (2000) categorized exceptions into nine areas (listed from the most to the least frequently cited in state statutes):
  - Complaints filed include after a counselor and client have had a dispute about another matter (such as billing or disagreements)
  - When a client is litigating their mental condition in a lawsuit
  - When a client might be a danger to self or others
  - When a report of child abuse has to be filed
  - When a client is contemplating committing a serious crime that might harm others
  - When a client is ordered to have a psychological examination by a judge
  - When a legal proceeding is held to determine if a client needs to be involuntarily hospitalized
  - When a client has been a victim in a crime
  - When a client has been accused or harming a vulnerable adult

Privilege belongs to clients not counselors

- It is up to the client to assert privileged communication unless:
- the client cannot be located, then the counselor should assert privilege
- If the client is deceased, the individual's legal representative should assert privilege
- See case example: video 5.1 caught in the middle

Which relationships enjoy privilege in communications in WA state?

https://doh.wa.gov/licenses-permits-and-certificates/professions-new-renew-or-update/social-worker-and-social-worker-associate/counselor-client-privilege-information#:~:text=Providers%20may%20not%20disclose%2C%20or,counseling%20service%20is%20considered%20privileged.

- Doctor/Patient (RCW 5.60.060(4))
- Lawyer/Client (RCW 5.60.060 (2))
- Priest/Penitent (RCW 5.60.060(3))
- Spouse/Spouse (RCW 5.60.060(1))
- LMHC, LMFT, LCSW/Client (5.60.06(9))
- Sexual Assault or DV Advocate (5.60.06(8&9))
- AA Sponsor (5.60.06(10))
- Psychologist/Patient (18.83.110)

DURING INTERNSHIP/PRACTICUM COUNSELING STUDENTS HAVE THE SAME ETHICAL OBLIGATIONS TO UPHOLD CONFIDENTIALITY AS A LICENSED COUNSELOR

Sharing information with other professionals and administrative staff

When administrative staff handle confidential information, it is accepted that privilege extends to these individuals

Counselors are encouraged to consult and get supervision. Supervision is required in the early years of counseling. Include the fact that you are getting supervision in your informed consent document (ACA F.1.c). Client identity is usually shared in supervision and clients should be aware.

Make every effort to protect the identity of your clients in consultation. If you cannot, obtaining client consent is a good rule

Get client consent for sending referrals or getting past records from other professionals

FYI- WA state RCW's pertaining to counselors

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RCWs > Title 18 > Chapter 18.225

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Chapter Listing

Chapter 18.225 RCW

MENTAL HEALTH COUNSELORS, MARRIAGE AND FAMILY THERAPISTS, SOCIAL WORKERS

Sections

HTML	PDF	18.225.005	Findings.
HTML	PDF	18.225.010	Definitions.
HTML	PDF	18.225.020	Misrepresentation—Licensed by department.
HTML	PDF	18.225.030	Limitation of chapter.
HTML	PDF	18.225.040	Secretary's authority.

https://app.leg.wa.gov/rcw/default.aspx?cite=18.225&full=true#18.225.105

Activity

1. Your group will receive a case study
2. Each group member should take a different role as stakeholders/members of a panel (examples may be the counselor, a teacher, a doctor, a lawyer, a supervisor, a peer consultant, a parent (if applicable), an agency director, a CPS representative, psychiatrist, etc-be creative) who will prepare answers regarding the **legal and ethical issues** raised in their assigned case.

1. Panel Question and Answer Session: 5 - 7 minutes per group.

- One at a time, each panel will take their place in seats in front of the classroom.
- One panel member will introduce each member and give their expert credentials (be creative).
- Another panel member will explain or read the case to the class.
- Once the case has been read, the panel provides their expert analysis and recommendations.
- The panel then asks if there are any questions the audience has regarding this case. The panel will answer the questions that another group has prepared regarding this case.